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Editor's Note: Addendum released July 4, 2007, and appended to the original judgment.

COURT FILE NO.: 02-CV-19463

DATE: 2007/04/30

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

CHRISTINA ABEL

Plaintiff

- and -

DALE HAMELIN

Defendant

) Colleen L. Burn and Laurie A. Tucker, for
) the Plaintiff

) Debbie Orth and Michael R. Switzer, for the
) Defendant

) **HEARD:** February 5-9, 12-16, 19-23
) and 28, 2007

REASONS FOR JUDGMENT

HACKLAND J.

Introduction

[1] This is an assessment of the damages suffered by the plaintiff Christina Abel (“Christina”) in a pedestrian-motor vehicle accident which occurred on the evening of January 20, 2000. Christina suffered relatively minor injuries in this incident, yet seven years later, at the time of trial, this 55-year-old woman is substantially disabled from a combination of a chronic pain condition, depression and significant cognitive impairment, the causes of which are vigorously contested. General damages are claimed as well as past and future loss of income and future housekeeping expenses.

The Plaintiff Pre-Accident

[2] Christina was born in Eritrea in May of 1951 and lived there and in Ethiopia until she came to Canada at the age of 41 in February 1993. She trained in Eritrea as a nursing assistant and after seven years in that role, she obtained her full nursing qualification in 1986 following two years of training in Addis Ababa. She completed this training in the midst of a civil war and under harrowing conditions. She also learned English as part of her training and became fluent in Italian. After working as a pediatric nurse in Addis Ababa for seven years, she accompanied her half-brother’s children to Canada. She sought and was granted asylum here. Christina has never married. She initially lived in Ottawa with her brother and did domestic work for him. She formed strong ties with a local church. Friends from this congregation assisted her with immigration issues and continued to provide her with social support during the seven-year period between her arrival in Ottawa and her accident as well as in the subsequent seven years, up to the present time.

[3] Christina was a busy and motivated person prior to the accident. Shortly after her arrival, she volunteered at Saint Vincent Hospital helping to feed chronically ill patients, accompanying them on organized excursions, assisted with the laundry and with bed-making. At the same time, through her church, she helped elderly persons with cleaning and cooking. In late 1993 she secured a job at a psychiatric care institution, at a minimum wage rate, performing the functions essentially of a personal support worker. She enjoyed this work and continued some of her volunteer activities. She also took English language training in the hope of some day writing her licensing examinations in order to attain an Ontario registered nurse designation.

[4] In 1999 Christina was hired as a personal support worker at Villa Marconi, a senior citizens residence in Ottawa for persons of Italian heritage. Initially she worked part-time, but soon progressed to full-time status, earning \$12.00 per hour. Her duties included doing the laundry, changing beds and personal care work with residents. There were some physically demanding aspects to this work; she described it as “a hard job”. She worked the night shift, 11:00 p.m. to 7:00 a.m. She enjoyed this work and enjoyed conversing in Italian with the residents. She held this job on January 20, 2000, the date of the accident. She has not worked since and as described later in these reasons, I find that due to her current medical condition she will not work again in any remunerative capacity.

[5] Christina inquired of the Ontario College of Nurses as to how she could qualify to write the licensing examinations for the registered nurse (RN) designation. The College reviewed her Ethiopian nursing transcripts and identified deficiencies in her clinical experience. They recommended that she take an upgrading program through a community college and upon successful completion, she could write the RN licensing examinations. Christina decided to pursue this training through Algonquin College in Ottawa. She first took ESL (English as a Second Language) training. She struggled with this, failing some courses, but eventually succeeding. In addition to her studies, while working at Villa Marconi, Christina maintained an active social life through her church. She resided with her 18-year-old nephew in a two-bedroom apartment near her work. She did most of the house work and cooking.

[6] Christina appeared to be in relatively good health in January 2000. She had had malaria and typhoid fever, appendicitis and surgery for a goiter, all in Eritrea. There was a complication related to the thyroid surgery. She did not wake up for a period of time after the surgery and she recalls the doctor telling her that there had been a problem with the anesthetic. This incident in her medical history formed the basis for some doctors speculating that possibly she had suffered a respiratory arrest that may have resulted in an hypoxic injury to her brain. This speculation arose in the context of certain doctors attempting to explain the etiology of Christina’s serious cognitive impairment which was first recognized during her post-accident assessments.

[7] After coming to Canada, Christina experienced a malignant lesion on her eyelid that was successfully treated with radiation therapy. In 1999, she suffered bruising to her lower back when someone dropped a parcel on her on an OC Transpo bus. She was examined and x-rayed in hospital and there were no fractures. She took several days off work and remained with some discomfort for a brief period.

[8] The first and only signs of pre-accident cognition problems appeared in the evidence of Christina's family doctor, Dr. Rosenbloom. She noticed that Christina seemed to be slow and confused and often unable to comprehend the medical advice she was receiving, notwithstanding her nursing training. She could also get confused about appointments and arrive a day late. Difficulties were identified with communication and compliance. Dr. Rosenbloom at the time attributed the problems to linguistic and cultural factors.

The Accident and Initial Medical Attendances

[9] At about 10:00 p.m. on January 20, 2000, Christina was on her way to work at the Villa Marconi. She had stepped off the bus and was crossing Merivale Road at Baseline in a crosswalk, with the walk signal. She got about half way across the crosswalk when she came into contact with the defendant's vehicle. She explained in her evidence that she does not really remember the impact. She spoke of the car "sticking to her". She recalls the driver picking her up and that she felt shaken up and did not want to move. She recalls being helped to a lady's car and being driven to a nearby Tim Hortons restaurant. Someone called an ambulance. She recalls waiting at the restaurant for about an hour because the ambulance had been misdirected. While waiting she spoke to a police officer, to the driver of the car that hit her and to a friend she had telephoned, as well as to her nephew.

[10] In his evidence, the defendant said that he was making a left turn onto Merivale through a gap in oncoming traffic and saw the plaintiff in the crosswalk at the last minute. He stated that he brought his car to a halt in time and he denies hitting the plaintiff. In cross-examination, he conceded that he might have nudged her. He said that he and a witness helped Christina walk the rest of the way across the street where she got into the witness' vehicle on her own. The witness,

Ms Chaplin, felt that there was an impact between the defendant's vehicle and the plaintiff. She recalls Christina rolling onto the hood of the car with her arms outstretched. She then rolled back onto her feet and, although unsteady, did not fall onto the ground. Ms Chaplin said that Christina appeared to be in shock. She said Christina walked with great difficulty the rest of the way across the street and got into her (Ms Chaplin's) car. Christina kept repeating "God bless you", as Ms Chaplin drove to the nearby Tim Hortons. Christina was able to get out of the vehicle on her own and to go into the restaurant.

[11] I find as a fact that the defendant's car (a Chevy Nova compact) did hit the plaintiff before it came to a complete stop, and while it was a very low speed impact, it was sufficient to propel the plaintiff onto the hood of his car, at least with respect to her upper body. The plaintiff then rolled back onto her feet and down onto one knee on the ground, as the defendant had testified. The plaintiff was then able to walk to the vehicle of the witness, Ms Chaplin, with the help of Ms Chaplin and the defendant. I further find as a fact that the plaintiff did not suffer a concussion, loss of consciousness or other closed head injury in this incident. The evidence of the plaintiff, the defendant and the witness, Ms Chaplin, do not support the plaintiff having suffered a closed head injury, nor does the emergency room record for the Queensway Carleton Hospital (where Christina was taken from the restaurant), nor the evidence of her walking away from the accident and engaging in coherent conversation at the Tim Hortons restaurant.

[12] The Queensway Carleton Hospital record indicates that there was no loss of consciousness at the accident and that the plaintiff exhibited normal mental status upon examination in the emergency room. There was tenderness noted around the plaintiff's left hip and some pathology around her right knee. The clinical examination did not suggest any serious orthopedic injury, either to her head or back. Christina was discharged home from hospital with a prescription for Tylenol 3 for her pain. The next day, Christina attended the Holland-Carling After-Hours Clinic, exhibiting symptoms of soft tissue or sprain type injuries and possibly headache. Her right knee was swollen medially and her shoulders exhibited a reduced range of motion.

The Medical Evidence

[13] It is difficult to fully appreciate, even after hearing all of the medical evidence called at this trial, how it is that the plaintiff has become as significantly disabled as she currently is, and this on a permanent basis. Christina was only able to testify for a period of approximately two hours at any one time during the trial and accordingly her testimony was dispersed throughout the trial. She was unable to stay in the courtroom other than during her testimony. She was clearly fatigued and easily confused. A number of medical witnesses, including physicians, psychologists and occupational therapists, testified as to Christina's progress over the seven years since the accident. Unfortunately, her level of functioning has deteriorated significantly over that period of time due, as I find, to a combination of musculoskeletal problems caused primarily but not exclusively by the accident of January 2000, depression over her chronic pain and her poor quality of life, and an overarching cognitive impairment which prevents Christina from effectively remediating her physical complaints and from developing any insight into or effective coping mechanisms to deal with her cognitive limitations.

[14] Christina's chronic pain syndrome, in particular, is primarily the result of injuries sustained in the accident. Her inability to remediate her situation seems to be the result of her cognitive limitations which, while not caused by the accident, I find to be substantially exacerbated by the accident and its aftermath.

[15] Christina's life currently revolves around attempting to deal with her chronic pain. Each week day afternoon is devoted to one of several therapies, either aqua therapy or physiotherapy. Her level of activity was described as "sub-sedentary", by one of the numerous occupational therapists who have attempted to assist her. Her aqua therapy and physiotherapy appear to be more of an activity than an effective therapeutic exercise, in that she is unable to exert herself. She is however extremely dedicated to these activities and often puts up with significant transportation difficulties in order to attend. My observation of Christina through the course of the trial was that she often appeared to be confused, became quickly exhausted and moved very slowly and carefully as though in considerable pain. None of the medical witnesses who testified at this trial suggested that Christina's pain or her limitations were anything other than genuine. Several of the occupational therapists had spent considerable time with Christina. Christina is partly dependent on the assistance

of her niece or friends from the church in relation to her shopping activities and medical attendances and other excursions. She currently lives by herself in a small one-bedroom apartment which she is able to maintain in terms of light housekeeping, but with one day's help with more demanding tasks from her niece, every two or three weeks.

[16] The defendant's expert, Dr. Matthew Faris, a physiatrist, provided the most careful and helpful analysis of Christina's medical condition. He assessed the plaintiff and studied the voluminous medical and other records which have accumulated over the seven years since the accident as well as Dr. Rosenbloom's (her family doctor) pre-accident office chart. When Dr. Faris examined Christina in February of 2004, he concluded that her physical complaints were "not that serious" and that other compounding variables had come into play. He noticed that her memory seemed to be a significant problem and that she had difficulties organizing her thoughts. In terms of her medical history, he noted that the incident following her thyroid surgery in Eritrea in 1991 may have been consistent with her suffering a hypoxic insult to her brain causing the type of diffuse brain atrophy which was identified on some of the brain imaging and neuropsychological reports which he had available to him.

[17] On his examination of the medical records, Dr. Faris was satisfied that Christina did not suffer a brain injury or any form of post-concussive syndrome in the accident. He was further of the opinion that the accident mechanism itself did not seem to be capable of causing serious musculoskeletal injury. He said there was a strain injury to the trapezius muscle from the accident and that this would reduce the range of motion in the neck and shoulders. Upon reviewing the family doctor's post-accident progress notes, he was of the opinion that the collision exacerbated the plaintiff's pre-existing back injury from the recent OC Transpo accident in which a heavy box had been dropped on her. This, when combined with Christina's degenerative problems in her spine, produced an evolving low back pain problem which, after a period of time, and in combination with the degenerative disc disease in her back, triggered an ongoing chronic pain process. He felt that the plaintiff exhibited a reasonable range of motion and that her chronic pain appeared to be in the "mild end" of chronic pain patients that he had seen. He did acknowledge however that the trigger point

injections which the plaintiff was receiving were likely a factor in temporarily ameliorating her symptoms.

[18] Dr. Faris stated that the plaintiff's impaired functioning could be explained on the basis of her chronic pain problems exacerbated by cognitive difficulties and psychological factors. He agreed that psycho-social factors played an important role, in particular that they amplified the chronic pain syndrome and thereby increased the patient's functional disability. He also noted that the longer a person has symptoms of chronic pain the less the likelihood of improvement and, in this case, agreed that the plaintiff's significant impairment was indeed permanent. Regrettably, Dr. Faris had not seen the plaintiff for a period of three years prior to the trial and therefore could not comment directly on what appeared to be a significant deterioration in the plaintiff's functioning by the date of the trial.

[19] The principal medical expert witness called on behalf of the plaintiff was Dr. Alex McKee, a psychiatrist who assessed Christina in March of 2005. I do not place as much weight on Dr. McKee's assessment of Christina as I do on Dr. Faris', primarily because Dr. McKee started from the proposition that Christina was subjected to a more significant trauma in the accident than I consider to have been the case and also because he was of the view that there was no interconnection between Christina's cognitive difficulties and her current chronic pain, a proposition which I do not accept. Nevertheless, I do accept Dr. McKee's opinion that Christina's chronic pain, and most of her current musculoskeletal complaints are the results of a process caused by the accident. Dr. McKee traced back the plaintiff's current headaches, low back pain and shoulder pain complaints to symptoms appearing shortly after the accident. Dr. McKee diagnosed a facet joint problem as being at the root of the plaintiff's cervical pain, whereas Dr. Faris did not accept that proposition. Nevertheless, the plaintiff is suffering from ongoing serious neck pain which had its origin in the motor vehicle accident in the opinion of both psychiatrists. Dr. McKee observed, correctly in my view, that there was minimal or no evidence of her current physical complaints existing prior to the accident and he therefore concluded that her ongoing problems and limitations were a direct result of the injury. He noted in his report:

Significant symptomatology has been documented from the time of the injury. Documentation from early after the injury including from the Canadian Back Institute noted headaches, neck pain, bilateral shoulder pain, mid-back and lower pain and right lower extremity symptoms. Significant symptoms were also noted by Mr. McGregor.

[20] Like most of the other clinicians, Dr. McKee felt that Christina suffered from a “complete inability to return to any reasonable employment.” He also was of the opinion that Christina sustained a “permanent serious impairment of important physical functions”, so as to meet the statutory threshold. This issue was appropriately not contested by the defence and, in my view, the evidence clearly establishes that the plaintiff’s musculoskeletal injuries, as they have evolved into a chronic pain syndrome, meet the statutory threshold requirement of the Bill 59 regime, (the *Automobile Insurance Rate Stability Act*), which was in force November 1, 1996 to October 1, 2003.

[21] Christina was also seen by Dr. Mendis, a neurologist, who first saw her in July of 2001 on referral from Dr. McGregor, a physiatrist who had been seeing Christina on behalf of the accident benefits insurer. Dr. Mendis also saw Christina in November of 2002 and in September of 2003. Dr. Mendis was of the opinion that Christina, although presenting a puzzling clinical profile, probably suffered a post-concussive syndrome from a closed head injury. He had no information which would clearly indicate that she had suffered a head injury, but drew that inference from her symptoms and the fact that she had been involved in a motor vehicle accident. On each of her visits Christina was subjected to some basic cognitive testing and seemed to score within normal ranges. For reasons which Dr. Mendis really did not explain to the Court’s satisfaction, he failed to attach any particular significance to the neuropsychological testing results which were available to him, which showed severe non-accident related cognitive problems. Dr. Mendis did say that if the plaintiff had a post-concussive syndrome from a minor closed head injury, he would expect the condition to ameliorate rapidly and not having seen the plaintiff since September of 2003, he was unable to comment on whether he would still adhere to his initial diagnosis.

[22] Dr. Mendis was of the view that part of Christina’s symptoms were related to post-traumatic stress and anxiety, secondary to certain ongoing cultural issues and to her frustration at her inability to succeed with her nursing studies. During the period when Christina was seen by Dr. Mendis, she was suffering headaches, back and neck problems and certain right leg complaints, which are

complaints which continue to date. It was apparent from the evidence of the rest of the treating team of medical professionals who were assisting Christina on behalf of the accident benefits insurer, that they were highly sceptical of Dr. Mendis' diagnosis of post-concussion syndrome, particularly because they were aware of the neuropsychological testing results and recognized the major extent to which cognitive problems were interfering with their efforts to provide Christina with helpful therapy. As noted previously, I am of the view that on the balance of probabilities Christina did not suffer a closed head injury or a post-concussive syndrome.

[23] Dr. Youssef, an anaesthesiologist specializing in the treatment of chronic pain, testified that he currently administers trigger point nerve block injections on a weekly basis to Christina. This gives her effective symptomatic pain relief for a period of three to five days. He said that she continues to suffer neck pain, shoulder pain and low back pain and described her condition as a "chronic pain syndrome". He said that he has no doubt that the plaintiff has psychological problems and is extremely pain focussed. He commented that generally with late stage chronic pain, the original insult becomes completely immaterial to the patient's suffering. On that basis, he declined to attempt to relate Christina's current symptoms to the motor vehicle accident.

[24] The Court was greatly assisted by the evidence of Dr. Tamra Ricci, a clinical psychologist, who was part of the original treating team assembled by the accident benefits insurer to assist Christina. Dr. Ricci immediately realized that Christina's concentration and attention appeared to be impaired. This was May of 2000, five months post-accident. Dr. Ricci stated, "I found her very confused and her concentration was a big problem during the assessment and I was concerned that that would also impede her ability to learn techniques and strategies in psychological treatment." She found Christina to be moderately depressed, lacking in insight, and exhibiting pain behaviours. Christina's prognosis was considered to be "guarded". Dr. Ricci referred Christina to a neuropsychologist, Dr. Judge, for a complete neuropsychological assessment. Dr. Ricci agreed with Dr. Judge's subsequent assessment that Christina's cognitive difficulties appeared to be unrelated to the motor vehicle accident. She went on to conclude in her report:

“Whatever their origin, her current neuropsychological difficulties will likely present as significant barriers to her rehabilitation progress. As well, they will likely present as obstacles to retraining and occupational pursuits.”

[25] Dr. Ricci described a very difficult therapeutic relationship with Christina. She said Christina made some gains, though it was “very, very slow”. Information had to be repeated to her numerous times and by the next session, Christina would have forgotten what was previously discussed. Christina had difficulty reading hand-outs and would be unable to remember the hand-outs that she had read. In February 1991, near the end of the therapy from Dr. Ricci, she observed that Christina “still presented with neuropsychological deficits in terms of this concentration and attention problem that was permeating all the areas of her life so it was making treatment difficult and it was also making it difficult for her to learn information in the academic setting.” Dr. Ricci was aware that Christina was attempting to further her nursing studies at Algonquin College but believed that she would not be able to complete the program from a cognitive perspective. She believed that Christina was “setting herself up for failure” because she lacked the cognitive ability to handle this type of academic program.

[26] Dr. Ricci’s services were re-engaged by the accident benefits insurer in 2006 in connection with a vocational assessment. The conclusion drawn by Dr. Ricci at that time, which I have no hesitation in accepting, is that Christina, through a combination of cognitive deficits and her chronic pain syndrome, is permanently unable to maintain any form of remunerative employment. The chronic pain prevented Christina from engaging in any type of physical work, particularly of the sort involved in a personal care assistant’s role and her cognitive deficits precluded her from any kind of supervisory or interpersonal roles, particularly if they were to involve exercising judgment or responding to potential emergencies.

[27] To the same effect is the opinion of Dr. R. Simard, a physiatrist who saw Christina in December 2006, shortly before the trial, and concluded that she currently suffers a complete inability to engage in any employment. At the same time she was seen by Dr. Suddaby, a psychiatrist, who diagnosed Christina as having “an adjustment disorder with depressed mood”. He offered the following helpful observation concerning the interaction between Christina’s chronic pain and her cognitive deficit:

Ms. Abel does report ongoing pain experiences. Certainly, her severe brain dysfunction would significantly impair Ms. Abel's ability to cope with her pain experiences and her ability to adapt to her chronic pain experiences. Severe brain dysfunction also likely impairs Ms. Abel's ability to fully benefit from the cognitive-behavioural interventions from Dr. Ricci. Certainly, Ms. Abel does not appear to have any uptake or insight as to the work that she is participating in with Dr. Ricci.

[28] There was extensive neuropsychological testing carried out, which followed the obvious cognitive impairment recognized by Christina's treating physicians after the accident. Dr. Judge assessed Christina on June 29 and July 20, 2000 on referral from Ms Deborah Pipe, who coordinated the therapy for the accident benefits insurer. Dr. Judge noted in his report that Christina had told him that "I don't think I hit my head (in the accident)" and he noted that she reported feeling a few seconds of confusion but no loss of consciousness and there were no marks on her head. Dr. Judge carried out extensive neuropsychological testing. He found that Christina functioned in the "extremely low range of cognitive and intellectual abilities for her age". Her verbal sub-tests occupied the borderline range and the performance sub-tests scores occupied the extremely low range. There was major impairment to her memory function. He stated at page 10 of his report:

The current neuropsychological findings indicate that Ms. Abel presently functions in the extremely low range of cognitive and intellectual abilities for her age. Measures of visuospatial construction and spatial span were performed in the extremely low range, as were measures of working memory.

[29] Dr. Judge did acknowledge that his interpretation of his findings concerning Christina was complicated by several factors including language, culture, the possibility of depression and the presence of chronic pain. He concluded that the current findings of diminished scores across all aspects of cognitive, intellectual, memory and executive function are global findings which are not consistent with the neuropsychological *sequelae* of mild traumatic brain injury, depression or chronic pain. Significantly, he was of the view that Christina's current neuropsychological findings "strongly suggest that she does not possess the required cognitive and intellectual resources, at this time, to participate in such a program". He was referring to Christina's planned enrolment in Algonquin College's full-time RN refresher program in September of 2000.

[30] Further neuropsychological testing was carried out in October 2001 by Dr. Claire Brandys, which showed results consistent with the testing carried out by Dr. Judge. At the same time as Dr.

Brandys' assessment, Christina was seen by Dr. Donna Ouchterlony, a rehabilitation specialist, who concluded her assessment with the observation that Christina's "current cognitive deficits are not the result of a neurodegenerative disorder and current cognitive deficits are likely related to anxiety, stress and pain levels. There exists the remote possibility that some generalized brain damages occurred at the time of Ms. Abel's respiratory arrest in 1991."

[31] In her evidence at trial, Dr. Brandys was asked whether a person, such as Christina, with significant cognitive deficits, would be more susceptible to suffer cognitive consequences to a traumatic event. Dr. Brandys responded as follows:

A. Again, yes, of a certain intensity, possibly. An individual who already has a pre-existing cognitive condition is more likely to have difficulties secondary to other cognitive factors. So for example, pain affects, comparing me, who I hope is a relatively unimpaired person to a person who had widespread cerebral dysfunction from an atrophy condition, that the same pain condition would be expected to probably have more effect with the atrophy. It's the concept of lower cognitive reserve, that there is less to begin with.

[32] Dr. Brandys went on to repeat that chronic pain could be more disabling to a person with cognitive deficits and more difficult to remediate or to teach such a person to understand and adopt remediative strategies.

Analysis and Conclusions

[33] I will summarize the Court's view of the medical evidence as a whole. I find that Christina is suffering from a major cognitive deficit which is non-accident related and in fact pre-existed the accident. However the cognitive problems were substantially exacerbated by the accident. The original cause of Christina's cognitive problem is unknown and it is unnecessary and unhelpful to speculate as to the cause(s). The family doctor's chart indicates that there were instances of confusion and non-compliance which hinted at cognitive problems before the accident. Christina however was able to carry on her employment as a personal support worker without any observed deficiencies. The cognitive deficit, I find, played a major role in Christina's inability to recover from the soft tissue injuries which she suffered in the accident and played a major role in the development of the chronic pain syndrome and associated depression and anxiety which have resulted in Christina's permanent functional limitations.

[34] It was the opinion of many of the consultants and treating physicians, as summarized previously, that Christina's accident related physical symptoms, when superimposed on her cognitive deficit, effectively prevented her from being able to accept, comprehend, and follow any effective remediation and promoted the development of her symptoms into a chronic pain syndrome. As Dr. Brandys indicated, the physical pain and anxiety exhausted Christina's limited cognitive reserves. This is what the law will recognize as a "thin skull" situation, i.e. one in which the accident victim suffers considerably more from her injuries incurred in the accident than would normally be the case, due to a pre-existing condition. When this occurs, the tortfeasor is responsible for the entire damages flowing out of the negligence, notwithstanding the pre-existing condition. However, the courts have recently reiterated, as discussed below, that the "but for" test remains the predominant test to establish causation in the law of negligence, such that a tortfeasor will not be responsible for injuries or losses which the victim would have suffered even without the negligent act. So in the present case, any cognitive limitations which Christina would have suffered had this accident not occurred and which would have eventually negatively impacted Christina's life, would not be compensable.

[35] In the recent case of *Resurfice Corp. v. Hanke*, [2007] S.C.C. 7, the Supreme Court of Canada reiterated that the "but for" test remains the basic test for establishing causation in the law of negligence. To the same effect is the recent decision of the Court of Appeal in *Aristorenas v. Comcare Health Services* (2006), 83 O.R. (3d) 282 at pp. 292-293, in which Rouleau J.A. emphasized that the "but for" test is the standard for establishing causation in negligence cases and the "material contribution" test is reserved for cases in which the "but for" test is unworkable. In *Resurfice Corp.* McLachlin C.J.C. stated at paragraphs 19-23:

¶ 19 The Court of Appeal erred in suggesting that, where there is more than one potential cause of an injury, the "material contribution" test must be used. To accept this conclusion is to do away with the "but for" test altogether, given that there is more than one potential cause in virtually all litigated cases of negligence. If the Court of Appeal's reasons in this regard are endorsed, the only conclusion that could be drawn is that the default test for cause-in-fact is now the material contribution test. This is inconsistent with this Court's judgments in *Snell v. Farrell*, [1990] 2 S.C.R. 311, *Athey v. Leonati*, at para. 14, *Walker Estate v. York Finch General Hospital*, [2001] 1 S.C.R. 647, [2001 SCC 23], at paras. 87-88, and *Blackwater v. Plint*, [2005] 3 S.C.R. 3, [2005 SCC 58], at para. 78.

¶ 20 Much judicial and academic ink has been spilled over the proper test for causation in cases of negligence. It is neither necessary nor helpful to catalogue the various debates. It suffices at this juncture to simply assert the general principles that emerge from the cases.

¶ 21 First, the basic test for determining causation remains the "but for" test. This applies to multi-cause injuries. The plaintiff bears the burden of showing that "but for" the negligent act or omission of each defendant, the injury would not have occurred. Having done this, contributory negligence may be apportioned, as permitted by statute.

¶ 22 This fundamental rule has never been displaced and remains the primary test for causation in negligence actions. As stated in *Athey v. Leonati*, at para. 14, *per* Major J., "[t]he general, but not conclusive, test for causation is the 'but for' test, which requires the plaintiff to show that the injury would not have occurred but for the negligence of the defendant." Similarly, as I noted in *Blackwater v. Plint*, at para. 78, "[t]he rules of causation consider generally whether 'but for' the defendant's acts, the plaintiff's damages would have been incurred on a balance of probabilities."

¶ 23 The "but for" test recognizes that compensation for negligent conduct should only be made "where a substantial connection between the injury and defendant's conduct" is present. It ensures that a defendant will not be held liable for the plaintiff's injuries where they "may very well be due to factors unconnected to the defendant and not the fault of anyone": *Snell v. Farrell*, at p. 327, *per* Sopinka J.

The question therefore must be asked: but for the motor vehicle accident of February 2000, would the plaintiff have suffered any of the damages or incurred any of the losses for which she presently claims?

[36] The medical evidence establishes that Christina currently suffers from an overarching cognitive impairment which is significantly affecting her life. This condition, while not caused by the accident, has played a major role in exacerbating the musculoskeletal problems which arise from the motor vehicle accident and which have resulted in a permanent chronic pain syndrome. However, in the Court's opinion it is an appropriate and necessary inference to draw from the medical evidence pertaining to the cognitive impairment, that even in the absence of the accident, Christina would at some point have had her life significantly affected by this condition. Her cognitive impairment on the evidence, is not in itself a rapidly progressing condition and it remains unclear as to exactly how the accident has accelerated the onset of this condition. Nevertheless I find that Christina's cognitive problems have been significantly exacerbated by her accident-related pain syndrome.

[37] The evidence establishes, in my view, that Christina's cognitive impairment, without reference to the accident, would have precluded her from achieving her goal of obtaining the RN designation in Ontario. I agree with Dr. Judge and Dr. Ricci in this regard. On the balance of probabilities, I find that Christina would not have passed her RN licensing examinations and she would instead have continued her career as a personal support worker. The evidence of Christina's post-accident academic struggles in the nursing refresher program at Algonquin College convince me that cognitive problems were the operative cause of her difficulties and would have prevented her from achieving her goal of obtaining her RN designation. I accept Christina's evidence that she had no interest in becoming a registered nursing assistant. Further, I find that the burden of her cognitive problems, when combined with the heavy work associated with her job responsibilities in the Villa Marconi, or other similar institution in which she might work as a personal support worker, would result in her retiring from the workforce at age 60, which in any event is the average retirement age for female workers in Canada and for health care workers in particular. But for Christina's non accident-related level of cognitive impairment, I would have accepted the plaintiff's submission that Christina would have worked to at least age 65 given her enthusiasm for her work, her late entry into the workforce in Canada and her financial circumstances.

[38] The financial calculations attendant on various assumptions concerning Christina's future earnings were canvassed in evidence by an actuary on behalf of the plaintiff and by an accountant called by the defendant. Wherever the evidence conflicted between these two witnesses, I prefer the evidence of the plaintiff's actuary, Guy Martel. The evidence of the defendant's accountant from the firm M.D. & D., a firm that provides calculations exclusively to the insurance industry, was unhelpful to the Court in several aspects. The M.D. & D. report used a methodology for the calculation of future pecuniary loss which was not in compliance with Rule 53.09. More seriously, the discount factors provided in this report in reference to the future loss of income calculation included a discount for workforce participation which, when combined with any reduction the Court might consider for retirement before the age of 65, would result in a duplicative and inappropriate reduction in the calculation of the plaintiff's future income loss.

General Damages

[39] I award Christina the sum of \$120,000, less the statutory deduction of \$15,000 in respect of her non-pecuniary general damages, for a net award of \$105,000. Seven years after the accident she suffers from chronic pain in her back, neck, shoulder and leg and her cognitive deficit, although pre-existing the accident, has become a major disabling factor in her life and her chronic pain and depression have combined with her cognitive problems to bring this about. Her quality of life has been seriously and permanently impaired. This award reflects a reduction for the fact that Christina's cognitive problems would by the time of trial, have begun to be a more significant problem in her life, even had the accident not occurred.

Past Loss of Income

[40] At the time of the accident in February of 2000 Christina earned \$12.00 an hour as a health care aid, working 75 hours in each 14-day period. There was a debate at trial as to what level of benefits she should be accorded. There was no evidence of the employer paying any benefits. The defendant's position is that Christina should be assumed to have 4% benefits representing statutory vacation pay. The plaintiff urged a figure of 13% which is based on information from the Ontario Nursing Association as to the recommended payment rate in lieu of benefits in private nursing contracts. I do not think it appropriate to find that Christina would have earned the same benefit rate as a nurse, particularly in the face of there being no evidence that she was receiving any benefits from her employer at the time of the accident. I find that a 6% allowance for benefits would be appropriate in the circumstances. On that basis, Christina's loss of income to the date of trial is \$180,807.00 plus benefits of \$10,848.42 for a total gross income loss of \$191,655.42. I request the plaintiff's actuary to calculate the net income loss after deducting statutory deductions and applicable income replacement benefits received from the insurer.

Future Loss of Income

[41] I accept the actuary's calculation that Christina's salary, which I adjust for a 6% benefit, would be \$29,832.64 annually as of the date of trial and would result in a future income loss, present valued at \$123,140.18, applying my finding that Christina would have remained employed as a personal support worker until the age of 60, had this accident not occurred. In addition, Christina will incur a loss of Canada Pension Plan retirement benefits as a result of her early retirement from

the workforce. I request the actuary for the plaintiff to recalculate this amount reflecting the findings of fact which I have made in these reasons. In addition, I will also require a calculation of the net future loss of income, reflecting an appropriate reduction for income taxes, Employment Insurance premiums and Canada Pension Plan premiums. I request counsel's written submissions as to the appropriate treatment of the future income replacement benefits, based on my findings.

Housekeeping and Personal Support

[42] The evidence establishes that Christina needed and accepted the assistance of her niece after the accident, initially quite frequently, and laterally once every two to three weeks, for the purpose of assisting with the heavier cleaning in her apartment, cooking the Ethiopian food she enjoys, shopping and personal errands. Her niece was paid by the accident benefits insurer for this work in the first two years and thereafter her niece has been doing this on a voluntary basis. Christina also has the occasional help of members of her church. In my opinion, Christina requires approximately one half-day per week of assistance with respect to the activities noted. I would allow the sum of \$100.00 a week, which reflects the cost of a personal care worker for approximately 4.5 hours weekly. This allowance should extend to the age of 75 when I find that Christina is likely to move into an assisted care facility. The present value of this allowance is \$86,827.48. This level of need is demonstrably justified by the evidence of the several occupational therapists who testified and is the result of Christina's chronic pain syndrome, and not the result of the cognitive deficits. The cognitive problems may require further professional interventions but they are, on the evidence, not caused by the accident. The evidence also satisfies me that this allowance should be based on the hourly rate of a personal support worker, not merely a housekeeper, having in mind Christina's significant personal needs and limitations.

[43] The general and special damages herein will bear interest in accordance with the provisions of the *Courts of Justice Act*. Any disagreement between counsel as to these calculations can be brought to my attention.

[44] I request the plaintiff to provide me with her submission as to costs within 60 days hereof and the defendant to respond within 30 days of receipt of the plaintiff's costs submission and then allowing the plaintiff a further 14 days for reply.

Mr. Justice Charles T. Hackland

Released: April 30, 2007

COURT FILE NO.: 02-CV-19463

DATE: 2007/04/30

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

CHRISTINA ABEL

Plaintiff

– and –

DALE HAMELIN

Defendant

REASONS FOR JUDGMENT

Mr. Justice Charles T. Hackland

COURT FILE NO.: 02-CV-19463

DATE: By Written Submission

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

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)

CHRISTINA ABEL	)	Colleen L. Burn and Laurie A. Tucker, for
	)	the Plaintiff
	)	
	)	
Plaintiff	)	
	)	
- and -	)	
	)	
	)	
DALE HAMELIN	)	Debbie Orth and Michael R. Switzer, for the
	)	Defendant
	)	
	)	
Defendant	)	
	)	
	)	
	)	HEARD: By Written Submission

2007 CanLII 17185 (ON SC)

ADDENDUM TO JUDGMENT DATED APRIL 30, 2007

HACKLAND J.

[45] In paragraphs 40 and 41 of my reasons for judgment, I requested the plaintiff's actuary to perform certain calculations in accordance with my findings. I have now received those calculations as well as the written submissions thereon of both parties.

[46] I accept the actuary's calculation of the past loss of income after deducting from the total gross income loss, including benefits, of \$191,655.42, both the required statutory deductions and the applicable income replacement benefits received. This amount is \$3,944.00, which I award to the plaintiff. I have accepted the actuary's use of the annual average weekly earnings as found in Statistics Canada's "Ontario Health Care and Social Assistance" category, as reflecting the appropriate inflation factor, rather than the consumer price index. I also credit the defendant with the slight overpayment of income replacement benefits which occurred in 2000 and 2001.

[47] The loss of Canada Pension Plan retirement benefits as a result of the plaintiff's early retirement from the workforce (at the age of 60) is \$25,327.00, and she is accordingly awarded that sum.

[48] I accept the actuary's calculation of net future loss of income in the sum of \$123,140.00, which I award to the plaintiff. I accept and agree with the observation in his report that the practise, following *Jennings v. The Queen*, [1966] S.C.R. 532 is not to deduct income tax and statutory premiums from future income losses.

[49] Further, as required by section 267.8 of the *Insurance Act*, I order that the defendant be entitled to an assignment of the statutory income replacement benefits due to the plaintiff, up to her 60th birthday. I awarded a tort recovery to the plaintiff for future income loss based on the finding that she would have retired at age 60 if the accident had not occurred, see *Bannon v. McNeely*, 38 O.R. (3d) 659 (C.A.).

[50] In summary, I award to the plaintiff the following amounts, plus any applicable pre-judgment interest calculated in accordance with the *Courts of Justice Act*:

(a)	general damages	\$105,000.00 (net of the statutory deduction)
(b)	past loss of income	\$ 3,944.00
(c)	loss of CCP retirement benefits	\$ 25,327.00
(d)	future loss of income	\$123,140.00
(e)	future housekeeping expenses	\$ 86,827.48

[51] As to the costs submissions, I vary paragraph 44 of my reasons for judgment and direct the plaintiff to submit her written costs submissions within 14 days of the release of this addendum, the defendant to submit his written submissions within 30 days of delivery of the plaintiff's submissions and the plaintiff may reply within 14 days of delivery of the defendant's submissions. The parties have indicated an interest in making oral submissions as well. This can be arranged through the trial coordinator after the written submissions have been delivered.

Mr. Justice Charles T. Hackland

Released: July 4, 2007

COURT FILE NO.: 02-CV-19463

DATE: By Written Submission

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

CHRISTINA ABEL

Plaintiff

– and –

DALE HAMELIN

Defendant

**ADDENDUM TO JUDGMENT
DATED APRIL 30, 2007**

Mr. Justice Charles T. Hackland

Released: July 4, 2007

COURT FILE NO.: 02-CV-19463

DATE: 2008/01/14

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

CHRISTINA ABEL

Plaintiff

- and -

DALE HAMELIN

Defendant

)
)
) Colleen L. Burn and Laurie A. Tucker, for
) the Plaintiff
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) Debbie Orth and Michael R. Switzer, for the
) Defendant
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CORRIGENDA

HACKLAND J.

[52] Paragraph 38 of the Reasons for Judgment herein dated April 20, 2007 are amended to replace the reference to Rule 57, with Rule 53.09.

[53] Paragraph 5 of the Costs Endorsement herein dated November 14, 2007 is amended so that the 5th and 6th sentences state:

But, as noted above, the trial judgment requires a set off or assignment of the plaintiff's future IRB's, the value of which an actuary has calculated to be \$63,485.00. Therefore the plaintiff's net recovery is \$344,238.48 + \$15,000.00 — \$63,485.00 = \$295,753.48.

Mr. Justice Charles T. Hackland

Released: January 14, 2008

COURT FILE NO.: 02-CV-19463
DATE: 2008/01/14

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

CHRISTINA ABEL

Plaintiff

– and –

DALE HAMELIN

Defendant

CORRIGENDA

Mr. Justice Charles T. Hackland

Released: January 14, 2008